A Phase 2, Open-Label Study with Orally Administered CVM-1118 and Sorafenib in Subjects with Advanced Hepatocellular Carcinoma

Protocol Number: CVM-004 Version 5.0

Statistical Analysis Plan (SAP)

Version 2.0: July 21, 2022

Document History

Date	Version Number	Reason of Revision	Revised By
August 28, 2018	0.1	First Draft	Jain Chung
October 31, 2018	0.2	• Revised based on comments and discussions at the "End of First Draft" meeting and SciQuus comments. • Add diagram to illustrate the dose escalation during the combination treatment period • Add more details on PD-efficacy and safety relationship analyses	Jain Chung
April 08, 2022	0.3	Revised and simplified based on the actual status of enrollment of the study.	Chia-Ying
April 15, 2022	1.0	Final version	Chia-Ying
July 21, 2022	2.0	• Revised the time window for Table 1. • Removed the standardization process of clinical laboratory data as described in section 6.8.2, and added narratives in paragraphs to explain how to handle the lab unit discrepancies.	Chia-Ying

Approval Page

This document has been reviewed and accepted as the approved Statistical Analysis Plan of Protocol Number: CVM-004.

This signed document is effective from the last signature date for the length of the indicated protocol or until replaced with a revision of the document.

Approved by:	Yen-Ling Chen	yenlingchen	07/21/2022
	Senior Manager of New Drug Development TaiRx Inc.	Signature	Date (mm/dd/yyyy)

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1. Introduction

This document is intended to provide comprehensive details on all statistical analysis methods and data handling rules to be used for analyzing all the data collected from First Patient's First Dose (FPFD) of CVM-1118+sorafenib (i.e., start of combination treatment) to Last Patient First Dose plus 24 weeks (LPFD+24 Weeks). The sorafenib tolerability assessment period (cycle 0) will be analyzed separately focusing on safety and tolerability only.

This document is a living document which may be revised during the study period and will be finalized prior to the database lock for incorporating changes in data handling rules or data analysis if necessary.

2. Objectives

This study is a Phase II, single-arm, multi-center, open-label study designed to evaluate the efficacy and safety of CVM-1118 combined with standard dose sorafenib in subjects with hepatocellular carcinoma. It will be conducted in US and Asia.

The Primary Objective is to evaluate the efficacy of HCC subjects treated with CVM-1118 and sorafenib standard dose combination (CVM-1118+sorfenib).

The Secondary Objectives are to

- Evaluate the safety profile of CVM-1118+sorafenib in HCC subjects
- Characterize pharmacokinetics (PK) of CVM-1118 and its metabolite CVM-1125 (TRX-818M1) when administered with standard dose sorafenib
- Explore any potential relationships between pharmacokinetics (PK) parameters of CVM-1118 and/or its metabolite CVM-1125 (TRX-818M1) with efficacy and safety endpoints.

3. Study Design

Subjects initially receive sorafenib alone as a single agent for a 3 weeks period at the approved dose and schedule (i.e., Sorafenib Tolerability Assessment period, termed "Cycle 0"). Following determination of an acceptable dose of sorafenib for each subject, the subject will then receive CVM-1118 in addition to sorafenib as illustrated in **Figure 1**. Subject who are unable to tolerate sorafenib will not receive CVM-1118 and will be replaced.

The first 3 subjects entered on study will receive a reduced starting dose of CVM-1118 of 150 mg BID. In the event that no Grade 3 or greater GI toxicity is observed in these 3 subjects during the initial 28 days of CVM-1118 and sorafenib, then an additional 3 subjects will be enrolled at a CVM-1118 dose of 200 mg BID with sorafenib. If no GI toxicity of Grade 3 or greater severity are observed in this second group of subjects receiving 200 mg, PO, BID of CVM-1118, then subsequent subjects may be enrolled at 200 mg BID of CVM-118 with sorafenib with no limits on the rate of enrollment (See **Figure 2**).

Once on study, subjects may have their dose of either or both study drugs reduced to address study drug-related toxicity with the goal of maintaining the dose of sorafenib at or close to the approved dose of 400 mg PO BID.

Figure 1: Overall Dosing Schema for Individual Subjects

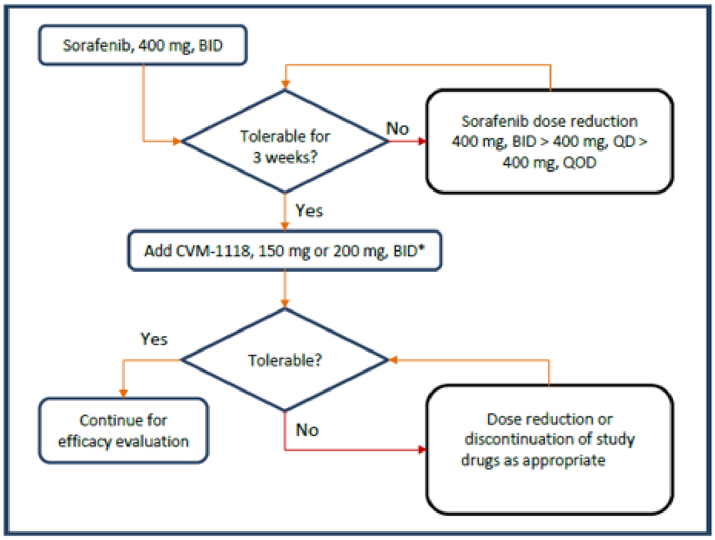
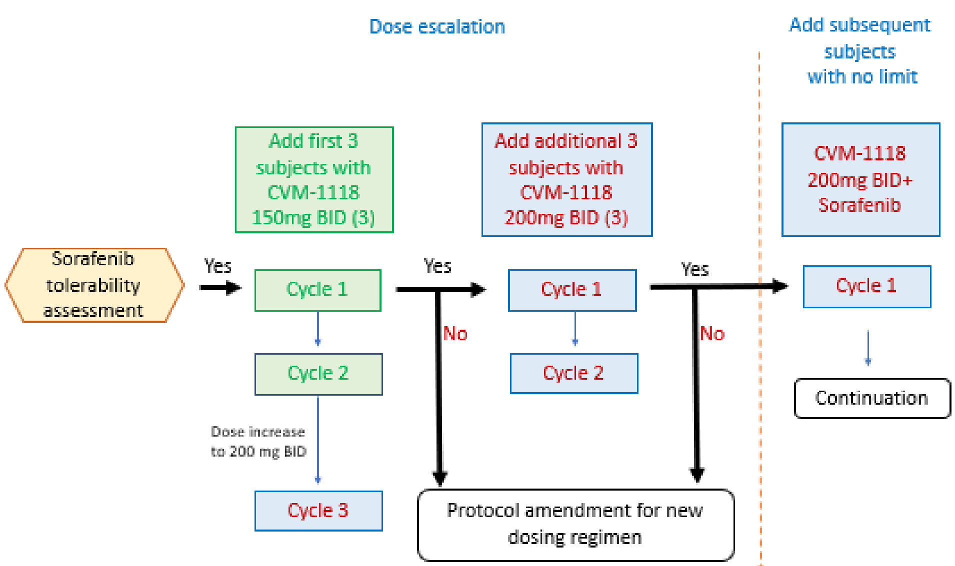


Figure 2: Initial Dose Escalation within the Combination Treatment Period



<u>Disease status</u> will be assessed <u>at the screening prior to the start of sorafenib</u>, at the start of [CVM-1118 + sorafenib] and <u>every 8 weeks</u> thereafter. Safety assessments will be performed using AEs, clinical laboratory values (hematology, coagulation, chemistries, and urinalysis), vital signs, and ECG.

Subjects who remain on study beyond Cycle 2 in the absence of significant toxicity will have the option to reduce the frequency of clinic visits to every two weeks.

PK analysis will initially be conducted in all subjects. The sponsor may elect to reduce or eliminate PK analysis performed for later subjects if appropriate.

Sample Size Calculation

The objective response rate (ORR) is used for the sample size estimation. The estimation is to ensure that the lower bound of the two-sided 95% confidence interval of ORR for patients treated with [CVM-1118 + sorafenib] will be above the response rate attributed by sorafenib treatment alone.

According to published studies, the range of response to sorafenib monotherapy is from 0 to ~10%. Assuming the ORR of sorafenib treatment alone is 4% and the targeted ORR of [CVM-1118 + sorafenib] treatment is 16%, a total of 36 subjects is required. In order to include an estimated 10% of dropout rate, a total of 40 subjects will be enrolled for the study.

A Safety Review Committee (SRC) will oversee the study conduct to ensure the safety of trial subjects.

4. Study Endpoint

4.1. Efficacy Endpoints

The clinical cut-off time point is 24 weeks (6 months) after the last subject's first dose of [CVM-1118 + sorafenib] (LPFD+24). The analysis may also be performed at other specified time points.

Modified RECIST criteria will be used to assess the responses. The sponsor may also elect to evaluate responses centrally using both the modified RECIST criteria and RECIST v1.1. If response evaluations are performed centrally, they will be conducted with a prospectively generated charter.

Primary efficacy endpoint

- Objective Response Rate (ORR): tumor reduction of a pre-defined amount (CR or PR as per modified RECIST) at clinical cut-off (LPFD+24 weeks) of [CVM-1118 + sorafenib] treatment

Secondary efficacy endpoints

Secondary efficacy endpoints will be evaluated at LPFD+24 weeks.

- Progression free survival (PFS) defined as the time (in months from the first dose of combination drug treatment (i.e., Cycle 1 Day 1) to the time of disease progression or death
- Time to progression (TTP) defined as the time from Cycle 1 Day 1 to the time of disease progression
- Overall survival (OS) defined as time from Cycle 1 Day 1 to death

- Disease control rate (DCR): defined as the sum of complete response (CR), partial response (PR), and stable disease (SD) rate as assessed by modified RECIST criteria
- Duration of response (DoR) defined as time from the first documentation of response (i.e., after the confirmation of initial response) to the time of disease progression

4.2. Safety Endpoints

Safety analysis will include all the data collected up to the clinical cut-off time point.

- Incidence (frequency and percentage) of AEs and SAEs
- Incidence of laboratory abnormalities and changes from baseline
- Incidence of ECG abnormalities (e.g., QTc interval)
- Changes from baseline in vital signs

4.3. Pharmacokinetics

Pharmacokinetic assessments: C_{max}, $C_{min(trough)}$, T_{max}, AUC_{0-8}, $T_{1/2}$, V/F, and CL/F for sorafenib, CVM-1118 and its metabolite CVM-1125 will be performed for all subjects entered on study during the following time points.

- Cycle 1 and Cycle 2
  - Day 1: pre-dose and 30 (±10) minutes, 60 (± 15) minutes, 120 (± 30) minutes, 3 (± 1) hours, 4 (± 1) hours, 6 (± 1) hours and 8 (± 2) hours post dose
  - Day 15: pre-dose (i.e., C_{trough})

- Cycle 3+ (and all subsequent cycles): pre-dose on Day 1 (i.e., C_{trough})

Time points for PK **sample drawing** are in relation to the dosing time of CVM-1118.

4.4. Pharmacodynamics analysis

Pharmacodynamics assessments will be performed between the main PK parameters (AUC and C_{max}) and the efficacy and safety endpoints for the study.

5. Analysis Populations

5.1. Safety Population (SAFP)

All subjects who received at least one dose of [CVM-1118 + sorafenib] with at least one follow-up safety assessment will be included in the Safety Analysis Population (SAFP). A follow-up assessment is defined as any vital signs, laboratory assessment, adverse event, or death reported after the first dose of study medication (i.e., CVM-1118 + sorafenib).

5.2. Efficacy Analysis Populations (EFFP)

All subjects who received at least one dose of study drug [CVM-1118 + sorafenib] and had at least one follow-up efficacy evaluation will be available for the efficacy analysis.

6. Statistical Analyses

This study is designed to generate preliminary efficacy and safety data of CVM-1118 co-administrated with sorafenib. The outcomes will be analyzed and summarized to assess the benefit and risk of [CVM-1118 + sorafenib] that would justify a confirmatory phase III study for further development.

The sorafenib tolerability assessment period (Cycle 0) will be analyzed separately focusing on safety and tolerability only.

6.1 Data Handling Rules

The first study medication date is defined as the date of the first co-administration of CVM-1118 with sorafenib (Cycle 1 Day 1). All study days are calculated from the date of the first medication. The last study medication date is defined as the last dose date of CVM-1118 recorded on the study medication page of the CRF.

Missing Dates

For efficacy analyses, missing dates are not acceptable and the data validation procedures in place for the study will ensure that complete dates are available for the analyses**.** For safety analyses, it will be specified along with the corresponding analysis later.

Missing Values

Patients without any post treatment assessment for efficacy or safety parameters will be excluded from the analysis of change from baseline for those parameters unless otherwise specified.

Subjects with missing assessment at scheduled visits (i.e. no data inside the time window of scheduled assessment) will not be imputed for the calculation of summary statistics at those visits.

Baseline and time windows for summary statistics

Time windows for summary of mean change from baseline are provided for each parameter listed in Table 1.

Table 1: Baseline and Time Windows for Summary Statistics over Time

Window Defined	Study Day	ECOG	Vital Signs	CBC with Differential	Serum Chemistries	Coagulation	Comments
Baseline C1, Day 1	1	(-2, 4]	(-2, 4]	(-2, 4]	(-2, 4]	(-2, 4]	Last value prior to Cycle 1, Day 1
C1, Day 8	8	(5, 11]	(5, 11]	(5, 11]	(5, 11]	(5, 11]	
C1, Day 15	15	(12, 18]	(12, 18]	(12, 18]	(12, 18]	(12, 18]	
C1, Day 22	22	(19, 25]	(19, 25]	(19, 25]	(19, 25]	(19, 25]	
C2, Day 1	29	(26, 32]	(26, 32]	(26, 32]	(26, 32]	(26, 32]	

C2, Day 8	36	(33, 39]	(33, 39]	(33, 39]	(33, 39]	-	
C2, Day 15	43	(40, 46]	(40, 46]	(40, 46]	(40, 46]	(40, 46]	
C2, Day 22	50	(47, 53]	(47, 53]	(47, 53]	(47, 53]	-	
C3$^+$, Day 1	57	(54, 60]	(54, 60]	(54, 60]	(54, 60]	(54, 60]	
C3$^+$, Day 8	64	(61, 67]	(61, 67]	(61, 67]	(61, 67]	-	Day 8 assessments may be eliminated
C3$^+$, Day 15	71	(68, 74]	(68, 74]	(68, 74]	(68, 74]	(68, 74]	
C3$^+$, Day 22	78	(75, 81]	(75, 81]	(75, 81]	(75, 81]	-	Day 22 assessments may be eliminated
EOT	**N**	Last value	Last value	Last value	Last value	Last value	Last value
SFU	**LD+28**	**(LD+21, LD+35**]	**(LD+21, LD+35**]	**(LD+21, LD+35**]	**(LD+21, LD+35**]	**(LD+21, LD+35**]	

The summary statistics will be reported up to 24 weeks of treatment (i.e., Cycle 6).

However, if the number of subject's data available is less than 3 after a Cycle (prior to Cycle 6) and onward, then the visits will only be showed up to the cycle with 3 or more subjects.

If there are multiple assessments within the same time interval, then only the worst assessment will be used for the calculation of summary statistics. Descriptive statistics will be based on actual observed data (i.e., missing data will not be imputed for the summary statistics).

6.2. Patient Disposition

Number of subjects who enroll into sorafenib tolerability assessment period (i.e., Cycle 0) and continue to receive combination treatment (i.e., CVM-1118 + sorafenib) and complete 24 weeks of CVM-1118 treatment will be tallied as follows:

Sorafenib tolerability assessment period

- Number of subjects entered into Cycle 0 (standard dose for sorafenib is 400 mg BID)
- Number of subjects with sorafenib stable dose of 400 mg QD, 400 mg QOD, other intermediate dose or withdrawn from the Sorafenib tolerability assessment period

Combination treatment period

- Number of subjects entered into Cycle 1 with CVM-1118 200 mg BID and with CVM-1118 150 mg BID
- Number of subjects withdrawn from combination treatment period
- Number of subjects completing at least 24 weeks of treatment
- Number of subjects entering long term follow-up

Number and percentage of subjects who discontinue the study by reason will also be provided. The reasons of withdrawal from the study are identified on the CRF study completion page.

6.3. Treatment Exposure

A Kaplan-Meier plot will be used to illustrate the timing of premature withdrawals from combination treatment (i.e., start on Day 1 up to the clinical cut-off day). Data collected beyond the clinical cut-off date except the survival follow-up for overall survival will be censored. The requirement for Kaplan-Meier plot will be determined by the sponsor.

The extent of exposure to treatments based on the number of days actually on CVM-1118, sorafenib, or both (combination). The duration of treatment (study day) will be defined as the number of days between the dates of the first and the last dose of combination treatment taken (incomplete day will be count as a full day). Cumulative frequency using categories ≤ 7, ≤28, ≤ 56, ≤ 84, ≤ 112, ≤ 140, ≤ 168, and >168 days for sorafenib tolerability assessment period and combination treatment period will be provided. (Note: dose interruption will be ignored.)

In addition, summary statistics: number of subjects receiving treatment, mean, standard deviation, median, and range of exposure will be provided.

6.4. Protocol Violations

Number and percentage of subjects who had protocol violations will be provided by reason. Subjects could have more than one protocol violations. It will be counted in each reason. A listing of all protocol violations will be provided. For major protocol violation, each subject will be counted only once for calculation of the percentage (i.e., total number violators divided by the total number of patients and multiplied by 100%).

- Use of prohibited anti-cancer medications or therapies during the study

- Use of prophylactic medications (not instituted by SRC) to reduce toxicity associated with study drugs
- Lack of compliance on medications (CVM-1118 or sorafenib intake is less than 75%)
- Violate inclusion and exclusion criteria which likely affect efficacy or safety assessments
- Others

The compliance on medication will be calculated for CVM-1118 and sorafenib separately. The compliance calculation is based on the total number of days the subject took study medication divided by the total number of days the subject was in the study regardless of the amount of study medication taken on each day.

6.5. Pre-study and Concomitant Medications

Concomitant medications and medications taken prior to the starting study treatment will be summarized for all subjects in the safety analysis population. Medications are considered concomitant if taken during the treatment period. Prior medications are medications with the start date and/or end date before study drug date. Medications will be summarized by WHO Drug therapeutic class and generic medication name.

6.6. Demographics and Baseline Characteristics

Demographics (sex, race, ethnicity, age, weight, height) and baseline disease characteristics (such as ECOG score, Alpha-fetoprotein (AFP), Hepatitis status, HIV experience, Child-Pugh classification) at baseline (Cycle 1 Day 1) will be summarized <u>for safety population only</u>. However, if the number of subjects of efficacy evaluable population is less than the safety population by at least 5 or more, then additional summary tables of demographics and baseline disease characteristics will also be provided.

For continuous variables, summary statistics including number of observations (n), mean, SD (standard deviation), median, minimum, maximum, and 95% CI (confidence interval) will be provided. Frequency tables will be used for summarizing categorical variables.

6.7. Efficacy Endpoint and Analysis

The efficacy analyses will be based on Efficacy Evaluable Population as defined in Section 5.2.

The clinical cut-off time point for clinical study report is set at LPFD+24 weeks after the last subject starts [CVM-1118 + sorafenib]. The analysis will be done after the database is clean and locked. Data received after the clinical cut-off time point such as the <u>long term survival follow-up data</u> will be excluded from this analysis plan unless it occurs <u>prior to the LPFD+24</u> weeks.

<u>Primary efficacy endpoint (ORR)</u>

The ORR will be based on the response assessments from both target and non-target lesions according to the modified RECIST criteria and Table 2 below.

Table 2: Overall Response Assessment in mRECIST (see protocol appendix)

Target Lesions	Non-Target Lesions	New Lesions	Overall Response
CR	CR	No	CR
CR	IR/SD	No	PR
PR	Non-PD	No	PR
SD	Non-PD	No	SD
PD	Any	Yes or No	PD
Any	PD	Yes or No	PD
Any	Any	Yes	PD

mRECIST, modified Response Evaluation Criteria in Solid Tumors; CR, complete response; PR, partial response; IR/SD, incomplete response/stable disease; PD, progressive disease

The objective response rate is calculated by the number of subjects who had either a confirmed CR or PR best overall response (i.e., 4 weeks after the initial response) and without new lesions during the defined analysis period divided by the total number of subjects in the Efficacy Analysis Population. Number and percentage of responders along with a 95% confidence interval will be reported.

Secondary efficacy endpoint

Time-to-event data: Time-to Progression Free Survival (PFS), Time to Progression (TTP), and Overall Survival (OS).

The study day of premature withdrawals will be used for censoring the data if it occurs prior to LPFD+24 weeks or safety follow-up unless the status of the event of interest is known. Subject's survival data will be censored at LPFD+24 Weeks for the analysis if they are still alive at that time point.

Median time to the event, if estimable, will be estimated based on Kaplan-Meier plots along with the 95% confidence interval. In addition, the corresponding event rates at the clinical cut-off time point will also be estimated based on Kaplan-Meier plot.

In order to explore factors which may affect the time to event outcomes, a Cox regression analysis may also be performed to include hepatitis disease status and ECOG score as covariates or other variables as appropriate. However, the Cox regression will not be performed due to the limited number of subjected being treated in the study that would not result in meaningful interpretation.

Binary data: Disease control rate (DCR: CR+PR+SD). Defined as the proportion of patients with a best overall response of CR, PR, or Stable Disease (SD), based on the investigator's assessment per RECIST criteria v1.1.

The DCR will be provided.

Continuous data: Duration of response (DoR)

Duration of response is the total duration (number of days) of CR or PR response during the study period.

Descriptive statistics such as n, mean, standard deviation, median, range will be used to report the outcomes**.**

6.8. Safety Endpoints and Analysis

Safety analyses will be performed on Safety Population. Safety data collected during the treatment up to 28-days following the last administration of CVM-1118 or prior to the start of new treatment, whichever comes first will be utilized for safety analysis. Safety will be assessed on the basis of AEs, concomitant medication use, physical examinations, vital signs, clinical safety laboratory evaluations, and ECGs. All safety information will be provided in subject listings.

6.8.1. Adverse events

Adverse events (AEs) will be graded according to the NCI Common Terminology Criteria for Adverse Events (CTCAE) Version 5.0 and coded to preferred term (PT) and system organ class (SOC) using the most recent version of MedDRA for the entire study.

All AEs reported during the AE reporting period (inclusive of the 28-day post last dose of study drug period or until a new anti-cancer treatment is started) will be considered as treatment-emergent adverse events (TEAE) and will be summarized by incidence table (including frequency and percentage). Incidence rates will be summarized with all subjects treated in safety population as the denominator, unless otherwise specified.

The following event scenarios will not be counted as adverse events.

- Event end date occurred prior to the start of [CVM-1118 + sorafenib] or the onset dates recorded after 28 days of the last dose of CVM-1118
- Pre-existing conditions unless the condition worsens by at least one grade following the start of [CVM-1118 + sorafenib] administration
- Disease progression unless resulting in death within 28 days of the last dose of CVM-1118

All laboratory abnormalities requiring treatment (e.g., medication, transfusion) or a procedure (e.g. hepatobiliary stent placement for increased bilirubin) will be listed as an AE.

AE incidence rates will also be summarized by severity and relationship to study drug. Treatment-related AEs are those judged by the Investigator to be at least possibly related to the study drug. Deaths that occur within 28 days after the last dose of study drug are defined as on-study deaths.

Patients with multiple occurrence episodes of a specific event with different severity or relationships will be counted only once for overall AE summary. However, for summary of AEs by severity or relationship during the study, only the most sever or most closely related episode will be attributed to the treatment in those incidence tables.

Patients who experienced any AE, any serious adverse event (SAE), any treatment-related AE, any treatment- related SAE, any discontinuations due to an AE, and any deaths will be summarized by the following tables:

- Overall summary of AEs: All AEs by SOC and preferred term
- Summary of distinct patients with all adverse events (highest grade per subject) for subjects receiving at least 1 dose of CVM-1118 by System Organ Class, preferred term and severity
- Summary of distinct patients with adverse events related to both CVM-1118 and Sorafenib (highest grade per subject) for subject receiving at least 1 dose of CVM-1118 by System Organ Class, preferred term and severity
- Summary of adverse events related to CVM-1118 only (highest grade per subject) by SOC, preferred term and severity for subject receiving at least 1 dose of CVM-1118
- Summary of adverse events related to sorafenib only (highest grade per subject) by SOC, preferred term and severity for subject receiving at least 1 dose of CVM-1118
- Summary of adverse events related to CVM-1118 and/or Sorafenib only (highest grade per subject) by SOC, preferred term and severity for subject receiving at least 1 dose of CVM-1118
- Summary of severe adverse events (SAE) for subject receiving at least 1 dose of CVM-1118
- Deaths and cause of death

Patient listings of all Grades 3 or 4 AEs, all SAEs, AEs that led to study drug discontinuation, and all deaths will be provided as well.

Narratives will be written for the following subjects in the final summary of report:

- Subjects that died within 28 days of the last dose of study drug
- Subjects that discontinued study drug due to adverse events
- Subjects who had a serious adverse event (SAE)

Analysis of Sorafenib Tolerability Assessment period (Cycle 0)

Sorafenib tolerability assessment will focus on sorafenib related adverse events (i.e., AEs of special interest). The following two summary tables will be provided for the summary of summary of report.

- Summary of Study Specifically Defined Adverse Events during Sorafenib Tolerability Assessment Period (Cycle 0)

6.8.2. Laboratory tests

Frequency table will be used for presenting categorical outcomes of laboratory test results (normal, abnormal without clinical significance, and abnormal with clinical significance). Shift table will be based on comparing the baseline category to the worst post-baseline category during the study.

Results outside the normal range will be considered as abnormal without clinical significance unless the clinical significance box on the CRF was checked. Local site's normal range will be used for the classification. Missing baseline test result will be considered as 'Normal.'

If applicable, all laboratory test results should be standardized according to the normal ranges from selected reference site prior to the calculation of the descriptive statistics. Descriptive statistics including number of observed subjects (n), mean, standard deviation, median, and range may be provided for actual test results and change from baseline results by visit. However, due to the lack of the function of the EDC system to handle the input and verification for reference normal ranges, with the limited access and difficulties in extracting large quantities of normal range data from source documents generated by 17 laboratories, the standardization will not be performed for the study report.

Tables and listings that require the standardization will not be provided, whereas narratives will be written for subjects with abnormal clinically significant or non-significant values. A complete listing of all laboratory data will be provided without the normal ranges whereas the copy of normal laboratory ranges in pdf format will still be attached to the study report.

Discrepancies in Unit of laboratory tests have been found between the final raw data and the sourced reference ranges after the database lock. Given the fact that the EDC system was not able to internally perform the verification process with reference normal ranges and that the study is an early terminated study, the re-verification will be performed by TaiRx without unlocking the database. The listings with corrected units will be attached to the final report as well.

6.8.3. ECG and vital signs

Three categories: normal, abnormal without clinical significance, and abnormal with clinical significance will be used to classify the ECG status. Shift table will be used to summarize the change from baseline in ECG status during the study. In addition, the rate of abnormality will be calculated for QTcF. The abnormality is defined as QTcF > 480 msec.

Descriptive statistics including numbe of observed subjects (n), mean, standard deviation, median, and range will be used to summarize continuous data of ECG (Ventricular rate, PR interval, QRS interval, QT, and QTcF) and vital signs (blood pressure, heart rate, body temperature, and respiratory rate) for observed and change from baseline value over time.

6.9. Pharmacokinetic and Pharmacodynamical Analysis

Pharmacokinetics

Peak plasma concentrations (C_{max}), C_{trough}, time of peak plasma concentration (T_{max}), half-life ($T_{½}$), area under the curve (AUC), V/F, clearance (CL), and CL/F will be calculated for sorafenib, CVM-1118 and its metabolite CVM-1125 (TRX-818M1) during Cycle 1 and Cycle 2.

Summary of drug concentration and derived PK parameters will be provided based on scheduled time points up to Cycle 2 except for C_{trough} which will be summarized up to the clinical cut-off time point.

Pharmacodynamics

The relationship between the PK parameters and the primary endpoints- objective response rate (ORR), secondary endpoints and **Grade 3 or 4 AEs** (safety) during **24** weeks of treatment period will be evaluated for patients who completed first Cycle of the combination treatment. The parameters AUC and C_{max} are derived from either Cycle 1 Day 1 PK concentrations or the steady state (i.e., Cycle 2 Day 1 PK concentrations). The C_{trough} is collected on Day 15 of Cycle 1 and Cycle 2 pre-dose and Day 1 of all subsequent cycles.

Summary of PK parameters AUC, C_{max}, and C_{trough} will be provided for responders and non-responders separately. Same summary table will also be presented for patients with or without Grade 3 or 4 adverse events for to explore the relationship between PK parameters and safety. Further analyses may be performed if appropriate. However, the analysis will not be performed due to the limited number of subjects being treated in the study that would not result in meaningful interpretation.